

Octo

Anti-Dandruff Shampoo · 200 ml

A quadruple-action formula: two antifungals, a cytostatic and a keratolytic — combined with tea tree, neem and soothing oils at scalp-friendly pH 5.5.

4 anti-dandruff actives | pH 5.5 | Dandruff & seborrhoeic scalp

4
Anti-dandruff actives
Targets the yeast, the scaling and the itch

0.7%
Piroctone olamine
+ Zinc pyrithione 1%

2%
Salicylic acid
+ Selenium sulfide 0.5%

MECHANISM — WHY FOUR ACTIVES

- **Piroctone olamine 0.7%**
  Antifungal against Malassezia, the yeast central to dandruff and seborrhoeic dermatitis.[1,2]
- **Selenium sulfide 0.5%**
  Cytostatic — slows excessive scalp-cell turnover — with antifungal activity.[3]
- **Tea tree 1% · Neem 3% · Ginger**
  Botanical antifungals; tea tree oil shampoo significantly reduced dandruff vs placebo.[4]
- **Zinc pyrithione 1%**
  Antifungal/antimicrobial; reference active in dandruff shampoos.[2]
- **Salicylic acid 2%**
  Keratolytic — loosens and removes scales so the antifungals reach the scalp.[5]
- **Panthenol · Jojoba · Squalene · Menthol**
  Moisturise the scalp and hair, reduce dryness and give a cooling anti-itch effect.

CLINICAL EVIDENCE ON THE INGREDIENTS

Piroctone olamine vs ZPT[2]
Schmidt-Rose, 2011
Moderate–severe dandruff, split-head design · piroctone olamine-based shampoo showed **dandruff reduction comparable to 1% zinc pyrithione**; ~90% reported less itching after 4 weeks.

Selenium sulfide[3]
Danby, 1993 · RCT
246 patients with moderate–severe dandruff · selenium sulfide and ketoconazole shampoos were both **significantly superior to placebo** at all visits, with reduced itching.

Tea tree oil[4]
Satchell, 2002 · RCT
126 patients · 5% tea tree oil shampoo daily for 4 weeks → **41% improvement** in dandruff severity vs 11% with placebo; itch and greasiness also improved.

Treatment: 2–3 times weekly for 3 weeks

Prevention: once weekly · Lather, leave 3–5 min, rinse

1. Borda LJ, Wikramanayake TC. J Clin Investig Dermatol. 2015;3(2):10.
2. Schmidt-Rose T, et al. Int J Cosmet Sci. 2011;33(3):276–82.
3. Danby FW, et al. J Am Acad Dermatol. 1993;29(6):1008–12.
4. Satchell AC, et al. J Am Acad Dermatol. 2002;47(6):852–5.
5. Arif T. Clin Cosmet Investig Dermatol. 2015;8:455–61.